

RESULTS

Unit №131 Kyiv city, Kolomyisky lane, 17/31A
Ukraine

Comments:

Patient:

Zherlitsyna Anastasiia

Date of birth: 25.04.1976

Gender: Female

Requisition number:

Lab accession number: 993343822

Collection date, time: 03.01.2023 9:49

Test Name	Result	Units	Reference Range	Comments
Coronavirus (SARS-CoV-2), COVID-19, RNA, RT-PCR-qualitative Primary sample: naso-oropharyngeal swab Detection of N and RdRp, E genes	not detected (negative)		not detected (negative)	The RT-PCR method is certified according to the DSTU ISO 15189
Relative viral RNA load in the assay sample (E gene)	not detected (negative)	Cycle threshold (Ct)	not detected (negative)	5-27 - high viral load 28-34 - average viral load 35-40 - low viral load > 40 - not detected Cycles (Ct) - the number of PCR amplification cycles
Relative viral RNA load in the assay sample (N and RdRp genes)	not detected (negative)	Cycle threshold (Ct)	not detected (negative)	5-27 - high viral load 28-34 - average viral load 35-40 - low viral load > 40 - not detected Cycles (Ct) - the number of PCR amplification cycles

Conclusion:

The genetic material of SARS-CoV-2 virus was not detected in the biological material sample examined by the RT-PCR-qualitative/FTD (urgent) test method.

In case you suspect to have the infection due to the presence of clinical signs and symptoms consistent with COVID-19, or symptoms of acute respiratory infection, we recommend to consult your doctor for further advice and carry out safety measures (wear a mask, keep safe physical distancing).

This testing is authorized to detect the genetic material of SARS-CoV-2 virus and is not intended to assess the immune response. Antibody testing is recommended to assess the immune response.

DILA Help Desk Informational support: 0 800 217 887.



Head of the Laboratory of Molecular Genetic Analysis



Pokrovskaya T.O.



What is Ct and how to interpret the results of your test for COVID-19?

Modern RT-PCR diagnostics of COVID-19 at DILA laboratory allows to:

- get the answer if virus is detected in patient's sample?
- determine the viral load – that is, approximate number of viral genetic material in 1 ml of tested sample.

Viral load indicates the contagiousness of an infected person to others and it is determined by the number of amplification cycles (Ct). Amplification is the process of increasing the number of copies of viral fragments in a sample. With each Ct cycle, the number of copies doubles. The more viral particles in the sample, the faster they will be detected by the PCR test system. If there are few viral particles, more amplification cycles are required to detect them. That is, the lower the Ct, the greater is viral load and thus, the greater is the person's infectivity to others. If after the 40th cycle of amplification viral genetic material is not detected in the sample, the test result is considered as negative.

Clinical manifestations of the disease may be absent (asymptomatic course) at any level of viral load.

WHAT DOES YOUR RESULT MEAN?

Ct 5-27 High VL (viral load)	A person is infected with COVID-19, and is very contagious to others. You should see your doctor, regardless of the presence or absence of the disease symptoms (which may appear later), to assess your condition based on the combination of physical and laboratory examinations. It is recommended to get a "Comprehensive study of risk factors for severe COVID-19" done to forecast the risks of severe illness, if you have medical conditions and risk factors (age, underlying comorbidities). It is recommended to repeat the RT-PCR test in 10-14 days. To determine the presence of an immune response, we recommend to conduct a test for IgM or total antibodies in 7-10 days, IgG - in 2-3 weeks.
Ct 28-34 Medium VL (viral load)	A person is infected with COVID-19, and is very contagious to others. You should see your doctor, regardless of the presence or absence of the disease symptoms (which may appear later), to assess your condition based on the combination of physical and laboratory examinations. It is recommended to get a "Comprehensive study of risk factors for severe COVID-19" done to forecast the risks of severe illness, if you have medical conditions and risk factors (age, underlying comorbidities). It is recommended to repeat the RT-PCR test in 10-14 days. To determine the presence of an immune response, we recommend to conduct a test for IgM or total antibodies in 7-10 days, IgG - in 2-3 weeks.
Ct 35-40 Low VL (viral load)	A person is infected with COVID-19. It indicates a low concentration of viral genetic material which is typically associated with a lower risk of infectivity. It may occur during the recovery period or when the infection has "descended" into the lower respiratory tract, and the main amount of virus is located in the cells of the lung tissue. When the condition worsens (increased fever, shortness of breath, cough) a doctor's consultation is recommended to assess your condition. It is also worth conducting an RT-PCR test of sputum or bronchial lavage and "Comprehensive study of severe course of COVID-19 risk factors" complex. At the onset of the disease (symptoms may appear later) it is necessary to consult a doctor in order to follow-up your condition and repeat the RT-PCR test in 10-14 days. To determine the presence of an immune response, we recommend the IgM or total antibodies test in 7-10 days, IgG - in 2-3 weeks. You can also be an asymptomatic Covid-19 carrier.
SARS-CoV-2 RNA not detected	No SARS-CoV-2 virus was detected in the RNA sample. Most likely the person is healthy or has already recovered. There is no risk of infecting others. If there are symptoms, it's likely: • A person is infected by another ARI's pathogen; • The virus is not concentrated in the nasopharynx, but in lungs or intestines; • Resampling - repeated nasal swab taken after a short interval (on the same or the next day), which reduces the amount of biomaterial in the sample and can lead to negative test result. To verify the diagnosis, it is recommended: • to repeat the RT-PCR test in 1-2 days to clearly determine the phase of infection: the beginning or the end of infection; • to determine if the immune response was mounted, have a test for IgM or total antibodies in 7-10 days, IgG - in 2-3 weeks; • to perform a test to rule out other infections (Screening for respiratory infections, PCR, 21 indicators, FTD (highly sensitive); • in severe cases, RT-PCR tests should be performed to determine the virus from other biological material (sputum, bronchial lavage, feces).

For explanation of your test result or if you seek a qualified assistance to choose the right test, please contact DILA Informational support service by phone: 0-800-219-696.

Important! Doctors at DILA laboratory do not establish diagnosis or prescribe treatment, but can only confirm or refute doubts about your health state, and advise which specialist you should consult.

Your test result is positive if any coronavirus gene is detected.

For more information, please visit our website: dila.ua